INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Cemiplimab for advanced cutaneous squamous cell cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00812
Version 3a

Reference treatment cycle

Treatment stages

01 **Cemiplimab treatment**
Every three weeks

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

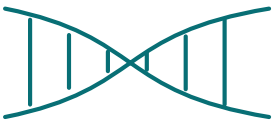
Medicines in this regimen

Cemiplimab helps immune cells act against advanced cutaneous squamous cell cancer when curative surgery or radiotherapy is not suitable. The team reviews cancer control, symptoms, side effects and your priorities to decide whether to continue.

PD-1 CHECKPOINT IMMUNOTHERAPY

Cemiplimab

Libtayo



Releases a PD-1 immune brake

Cemiplimab blocks the PD-1 immune checkpoint. This can help T cells recognise and attack skin squamous cell cancer, but increased immune activity can also inflame healthy organs.

PD-1 blocked → Immune response activated → Organ inflammation possible

What to watch for

Carry an immunotherapy alert card and report new diarrhoea, cough, breathlessness, rash, weakness or endocrine symptoms promptly.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use these questions with your own pathology report and agreed treatment plan.

Confirm that this is skin squamous cell carcinoma (SCC)

This pathway is for squamous cell carcinoma starting in the skin. Squamous cancers that begin in an internal organ follow different pathways. Keep the original skin site, pathology date and any sites of recurrence or spread in your record.

Why a cancer is considered high risk

The team considers the tumour's size, depth, site and involvement of nearby structures or nerves, along with recurrence and immune status. Ask which features affect your plan. A risk category is not a measurement of treatment response.



Why medicine is being offered

This cemiplimab pathway applies when advanced skin squamous cell carcinoma cannot appropriately be treated with curative surgery or radiotherapy. Ask the team to record the treatment aim and why this option fits your situation.

Immunity and transplantation

Tell the team about a transplant, autoimmune illness and all immune-suppressing medicines. These can alter the risks of immune treatment. Oncology and the relevant specialists need a shared plan; do not stop transplant medicines or steroids yourself.

Lesions, wounds and daily function

Ask for help with wound care, pain, bleeding, eating, movement or sleep. Keep a contact for worsening bleeding, infection symptoms, new nerve pain or weakness. These changes need assessment rather than waiting for the next photograph.

Your medicines and the overall plan

The schedule in this setting

Cemiplimab is given by intravenous (IV) infusion on Day 1 every 21 days. The usual source dose is 350 mg over 30 minutes; the prescription and visit checks confirm your treatment. There is no fixed cycle total in this regimen: treatment continues while the team considers it beneficial and tolerable.

The role of earlier or later treatment

Ask how the medicine fits with previous or planned local treatment. The timing of an operation, radiotherapy and a tablet course should be written in one agreed plan.

Pauses and stopping

A safety pause is an active treatment decision. Record which medicine is paused, the reason, the tests or recovery needed and who may authorise restarting. Do not restart from an old prescription or reset the history when a new phase begins.

Questions, decisions and practical support

Your priorities and the burden of treatment belong in the discussion.

Benefits and uncertainties

Ask what outcome this treatment aims to improve for your diagnosis and what alternatives exist. Results from a study group help a discussion but cannot promise your own result.

Alternatives and review points

Agree the next decision point and what would make the plan change. Ask about other suitable treatments, trials and symptom care. Support for comfort and function can continue alongside cancer treatment.

Help with daily life

Tell the team what is hardest in daily life: travel, work, caring duties, costs, sleep or confidence. Ask who can help and how to contact them.

Shared understanding

Before going home, check that you can describe your medicine schedule, next tests and urgent-contact route. Ask for an interpreter, larger print or a supporter if that would help.

What the evidence means for your treatment

Discuss the evidence for your diagnosis, tumour test results and treatment setting.

Ask which outcomes matter in your setting and what benefit is realistic for you. No survival percentage from another regimen, dose combination or disease setting is assigned here.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The overall treatment plan

Cemiplimab for advanced cutaneous squamous cell cancer

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY THREE WEEKS

Cemiplimab treatment

No preset cycle total **Cemiplimab infusion every 3 weeks; no fixed cycle total in this regimen.**

Cemiplimab is given by intravenous (IV) infusion on Day 1 every 21 days. The usual source dose is 350 mg over 30 minutes; the prescription and visit checks confirm your treatment. There is no fixed cycle total in this regimen: treatment continues while the team considers it beneficial and tolerable.

The treatment setting determines duration; a reference calendar is not a prescription or a reason to restart a held medicine.

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00812 v3a: current regimen source

THE WHOLE TREATMENT JOURNEY

Cemiplimab for advanced cutaneous squamous cell cancer: treatment and checks

NCCP 00812b: metastatic or locally advanced cutaneous squamous cell carcinoma when curative surgery or curative radiotherapy is not appropriate. This guide covers skin SCC, not the separate cervical-cancer indication.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00812	Full blood count, kidney/ liver tests, glucose and thyroid tests; hepatitis B surface antigen, hepatitis B core antibody and hepatitis C testing. Review autoimmune conditions, organ transplantation, previous immune reactions and medicines.	Full blood count, kidney/ liver tests and glucose each cycle; thyroid-stimulating hormone (TSH) every three to six weeks. Review new symptoms at every visit and arrange additional tests when indicated. Response assessment and scans follow the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Immune effects can begin or persist after treatment stops. Keep the agreed follow-up, steroid or hormone-replacement instructions and alert card, and tell clinicians about previous cemiplimab. The next scan, skin/ lesion review and any further treatment are individually planned.
Cancer, skin and daily function NCCP 00812	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up examines the treated area, other skin and relevant lymph nodes. The team adds scans or other specialist checks for your disease and symptoms. Keep wound care and skin surveillance in the same plan.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Medicines and continuing precautions NCCP 00812	Review all medicines and the pregnancy/ fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Use effective contraception during cemiplimab treatment and for at least four months after the last dose. Discuss fertility before treatment, and follow the medicine-specific breastfeeding restrictions. Report a possible pregnancy promptly.

What each check is for

1 Cemiplimab treatment

Every three weeks

Cemiplimab helps immune cells act against advanced cutaneous squamous cell cancer when curative surgery or radiotherapy is not suitable. The team reviews cancer control, symptoms, side effects and your priorities to decide whether to continue.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

Cemiplimab is infused into a vein over 30 minutes in the source schedule. Allow additional time for assessment, preparation and observation. Tell staff immediately about itching, flushing, chills, pain, dizziness, breathlessness or swelling during the infusion.

STEP 2

Missed, vomited or held treatment

If a visit is missed or treatment is held, contact the unit for a new appointment and treatment decision. Do not assume the next scheduled date means treatment has been cleared to restart.

STEP 3

Appointments and supply

Plan blood tests, travel and medicine collection in advance. If you need an interpreter or someone to help at home, tell the team before the appointment.

STEP 4

Paper copies and contact information

Ask for your latest schedule and contact details on paper. Replace old versions when treatment changes so everyone helping you uses the same plan.

Taking treatment and preparing for visits

Keep the written medicine plan beside this guide; it records your own dose and dates.

Taking or receiving the medicine

Other medicines and supplements

Show the pharmacist a current list of prescriptions, over-the-counter medicines, vitamins, herbal products and supplements. Include anticoagulants, heart medicines, steroids and recent antibiotics. Check before adding, stopping or changing anything; an apparently harmless product can alter treatment or confuse a symptom assessment.

Practical preparation

Bring your medicine list, allergy information and questions. Tell the team about practical problems with travel, swallowing, wound care or remembering doses before treatment starts.

Follow-up and your continuing record

Keep this record when treatment is paused, changed or completed.

Your follow-up plan

Follow-up examines the treated area, other skin and relevant lymph nodes. The team adds scans or other specialist checks for your disease and symptoms. Keep wound care and skin surveillance in the same plan.

Continuing medicine precautions

Immune effects can begin or persist after treatment stops. Keep the agreed follow-up, steroid or hormone-replacement instructions and alert card, and tell clinicians about previous cemiplimab. The next scan, skin/lesion review and any further treatment are individually planned.

Treatment summary for other clinicians

Record the medicines and last dates, important reactions, any long-term hormone replacement or steroid taper, and the responsible oncology contact. Show this information in urgent care and to other specialists. A different treatment phase does not erase earlier toxicity.

The complete paper version

Use this handbook as your paper record. Keep the current schedule, medicines, tests and contacts together, and replace outdated pages after a change. The matching QR link is optional.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Cemiplimab infusion	 **Day 1** Treatment after clinical review and the required tests.
Symptoms between visits	 **Days 1–21** Immune effects can begin at any time, including after treatment stops.
Next-cycle assessment	 **Day 21** The team confirms whether the next dose can proceed.

Timing note 1: Cemiplimab is given by IV infusion on Day 1 every 21 days. The usual source dose is 350 mg over 30 minutes; the prescription and visit checks confirm your treatment. There is no fixed cycle total in this regimen: treatment continues while the team considers it beneficial and tolerable.

Timing note 2: If a visit is missed or treatment is held, contact the unit for a new appointment and treatment decision. Do not assume the next scheduled date means treatment has been cleared to restart.

Timing note 3: This patient suite covers cutaneous SCC only; it does not transfer cervical-cancer treatment context.

Timing note 4: The cited NCCP indication is advanced disease unsuitable for curative surgery/radiotherapy; no adjuvant CSCC claim is made.

Timing note 5: A visible skin response does not exclude toxicity or establish response elsewhere.

Why these bands stay broad: This calendar shows the pattern. Your team supplies actual dates, prescribed doses and any hold or restart instructions. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00812 v3a: current regimen source ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1: Treatment visit

Cemiplimab is infused into a vein over 30 minutes in the source schedule. Allow additional time for assessment, preparation and observation. Tell staff immediately about itching, flushing, chills, pain, dizziness, breathlessness or swelling during the infusion. Report symptoms and previous reactions before treatment.

Day 2–21: Between visits

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy. Do not wait for the next infusion to report a possible immune effect.

Support around the medicines

1 **An individual symptom plan**

Ask which anti-sickness or other symptom medicines are prescribed, when to take them and when to call if they do not help. Do not start a borrowed medicine or alter treatment to manage a side effect yourself.

2 **Your urgent contacts**

Keep your actual oncology, out-of-hours and pharmacy contacts on the wallet card. Example numbers on a template must be replaced before you use it.

3 **Pregnancy and fertility precautions**

Use effective contraception during cemiplimab treatment and for at least four months after the last dose. Discuss fertility before treatment, and follow the medicine-specific breastfeeding restrictions. Report a possible pregnancy promptly.

4 **Eating, activity and practical help**

Let the team know if cramps, altered taste or poor appetite make eating or moving difficult. Ask for practical food and activity advice that fits your symptoms and any other illness.

5 **Steroids and hormone replacement**

Some immune effects need steroids or lasting hormone replacement. Follow the exact prescription and taper. Carry the relevant alert information, and do not stop or reduce these medicines yourself when anticancer treatment is held or completed.

Tests: what is required and what each result means

Safety tests, tumour tests and response scans answer different questions.

Before treatment

Full blood count, kidney/liver tests, glucose and thyroid tests; hepatitis B surface antigen, hepatitis B core antibody and hepatitis C testing. Review autoimmune conditions, organ transplantation, previous immune reactions and medicines.

During treatment

Full blood count, kidney/liver tests and glucose each cycle; thyroid-stimulating hormone (TSH) every three to six weeks. Review new symptoms at every visit and arrange additional tests when indicated. Response assessment and scans follow the individual plan.



Scans and tumour markers

Ask the team to write the next assessment and who will arrange it. Bloods used to monitor kidneys, liver, hormones or muscle are not automatically measures of cancer response. There is no universal tumour-marker blood schedule for these skin-cancer regimens; a scan or extra blood test is requested for a specific clinical reason.

Getting the result

Keep a named contact for bookings and results. If an expected appointment or result is missing, call that service; do not take silence as confirmation that everything is complete.

What to expect between visits

You can feel well and still need the planned checks. New symptoms deserve attention even on a day without an appointment.

Your usual pattern

Record the start and change in symptoms, temperature if advised, bowel pattern, drinking, urine and the effect on normal tasks. A simple few-line record is enough. Use it to describe a change rather than trying to grade or diagnose a toxicity yourself.

Eating, drinking and activity

Report nausea, mouth discomfort, diarrhoea or weight loss before they make eating and drinking difficult. The team can adjust symptom support and arrange dietitian or rehabilitation advice.

Skin checks, sun protection and optional photographs

This complements clinical assessment and does not replace it.

Protect your skin

Protect exposed skin with clothing, a hat and shade. Use broad-spectrum sunscreen with sun protection factor (SPF) 30 or higher and reapply as directed. Avoid sunbeds. Cloudy weather does not remove ultraviolet exposure.

Know the pattern of your skin

Ask the team to show you how to check your skin and treated area. Use a mirror or a helper for areas you cannot see. Report a new lump, a changing lesion or a sore that does not heal.

Photographs are optional

A private, dated photograph can help describe a change if your team finds it useful. Keep the lighting and size reference consistent. Photographs support a discussion; they do not replace examination and need not be uploaded.

Wounds, nails and sensitive skin

Follow the wound and skin-care instructions for the treated area. Ask before adding a cream or remedy. Increasing redness, discharge, pain, bleeding or difficulty using the area needs assessment.

Pregnancy, fertility and protecting others

Discuss these precautions before treatment even if pregnancy is not currently planned.



Medicine-specific precautions

Planning ahead

Ask early about fertility preservation, sexual health and symptoms affecting intimacy. Tell the team about any chance of pregnancy, breastfeeding or a partner's pregnancy so that the correct medicine-specific advice can be given. Keep the precautions and their end dates in the written plan.

Safe handling and supply

Do not share anticancer medicines. Keep oral medicines in their original labelled packaging, away from children and anyone who could take them accidentally. Return unused medicines to the supplying pharmacy and follow its handling advice. A treatment hold does not remove pregnancy precautions.

Questions worth asking

When are my next tests?

Full blood count, kidney/liver tests and glucose each cycle; thyroid-stimulating hormone (TSH) every three to six weeks. Review new symptoms at every visit and arrange additional tests when indicated. Response assessment and scans follow the individual plan. Your team adds the actual dates.

Can I use this without a smartphone?

Yes. The full handbook, three-page summary and wallet card are printable. The QR code is optional; keep actual contacts and written appointments on the paper copy.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

Your diagnosis and treatment setting NCCP 00812b: metastatic or locally advanced cutaneous squamous cell carcinoma when curative surgery or curative radiotherapy is not appropriate. This guide covers skin squamous cell carcinoma (SCC), not the separate cervical-cancer indication.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cemiplimab treatment

Active medicines: Cemiplimab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Routine antiemetic prophylaxis or premedication is not usually required by the source. Provide an immune-toxicity contact/alert plan; steroid or hormone treatment is prescribed only for the individual indication.
- Use effective contraception during cemiplimab treatment and for at least four months after the last dose. Discuss fertility before treatment, and follow the medicine-specific breastfeeding restrictions. Report a possible pregnancy promptly.
- Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hormones / head / eyes

Cemiplimab

New severe headache, visual change, unusual tiredness, dizziness, thirst or frequent urination may signal immune effects and needs prompt advice.

2 Lungs / breathing

Cemiplimab

New cough or breathlessness may reflect lung inflammation or another serious cause. Call promptly; severe breathing difficulty is an emergency.

3 Heart / muscles

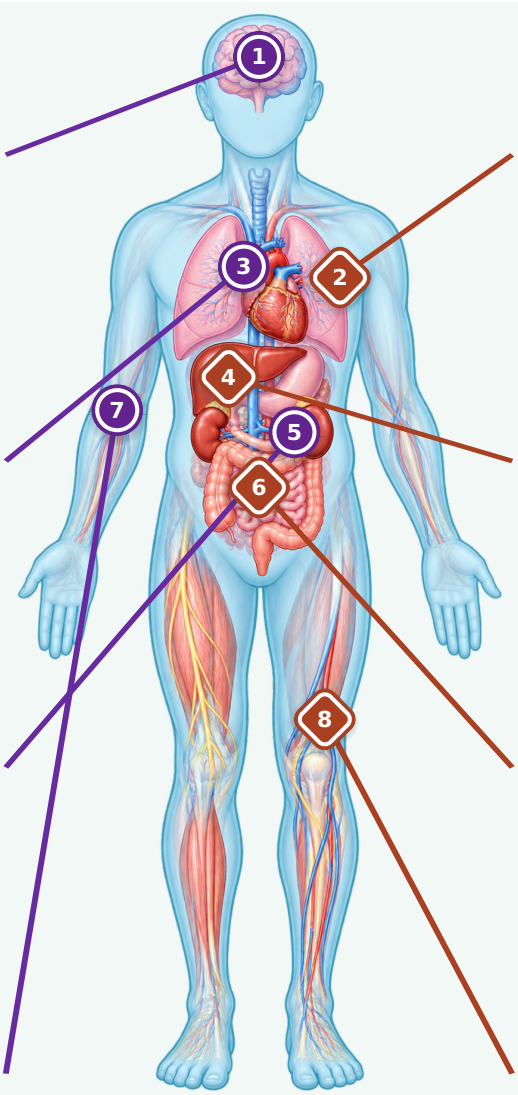
Cemiplimab

Chest pain, palpitations, marked muscle weakness or swallowing difficulty can be serious immune effects. Seek urgent assessment.

4 Liver

Cemiplimab

Inflammation may first show in blood tests. Report yellow eyes or skin, dark urine, unusual itching or upper-abdominal pain.



5 Kidneys / urine

Cemiplimab

Kidney inflammation may be detected on blood tests. Report reduced urine, swelling, blood in urine or a marked change from normal.

6 Bowels / abdomen

Cemiplimab

Report increased stool frequency, diarrhoea, blood or mucus, or abdominal pain early. Severe pain, black stools or dehydration needs urgent help.

7 Skin / mouth

Cemiplimab

Report rash or itching early. Blistering, peeling, mouth or genital sores, or rash with fever needs urgent assessment.

8 Nerves / strength

Cemiplimab

New tingling, weakness, unsteadiness, facial change or trouble swallowing needs prompt review. Sudden weakness or speech difficulty is an emergency.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00812 v3a: current regimen source**
   https://healthservice.hse.ie/documents/6426/812_V3a_Cemiplimab.pdf
2. **EMA: Libtayo medicine overview**
   https://www.ema.europa.eu/en/medicines/human/EPAR/libtayo
3. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
4. **NCI: palliative care during cancer treatment**
   https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet
5. **NCI: skin cancer — patient information**
   https://www.cancer.gov/types/skin/patient/skin-treatment-pdq
6. **NCI: skin cancer treatment — clinical evidence**
   https://www.cancer.gov/types/skin/hp/skin-treatment-pdq
7. **HSE SunSmart: protection from sun and sunbeds**
   https://www.hse.ie/conditions/cancer/protect-skin-sun-sunbeds/